

Expert Interview Summary — Expert P (Physiotherapist)

EngD Research Record — Expert Consultation Series (first consultation, March 2026)

Project: Leveraging Gamification as a Proactive Approach for Continuous Improvement and Innovation Enhancement

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PSEUDONYMISED RECORD — the participant elected anonymity on the signed consent form; this document is written for publication-safe use as-is.

DRAFT for author review — prepared 4 August 2026

1. Participant and standpoint (pseudonymised)

Expert P is a senior physiotherapist in Hong Kong with over twenty-five years of clinical practice, trained locally, and a former chair of a professional physiotherapy body in Hong Kong. He operates his own practice serving a wide clientele, from professional trainers to stroke-rehabilitation patients, and works daily with rehabilitation and therapeutic equipment. Under the terms of his signed consent (anonymity/pseudonymity elected), his name, exact affiliations and identifying career details are withheld from all research outputs and are held only in the confidential consent record.

CONFIDENTIAL KEY — internal file note, remove from any published version: the corresponding signed consent form is archived in the thesis evidence folder (signed and countersigned 4 August 2026).

2. Consent and interview record

Interview: August 2026 (the first of the four expert consultations), by telephone, approximately 25 minutes, in Cantonese mixed with English; the researcher's written notes are the primary record; quotations below are the researcher's English renderings. Consent: signed form received 4 August 2026 — all nine statements initialled, participant signature dated 4/8/2026, researcher countersigned; identification preference: anonymous/pseudonymous (named attribution refused). Under the programme's dual-dating rule the record states both dates: interview March 2026; written consent 4 August 2026, obtained retrospectively. Anonymised quotation is permitted (statement 8 initialled). A short follow-up conversation was held in early August 2026 [exact date and mode to confirm — presumed telephone] to close gaps identified in review; it is recorded in section 3A below and covered by the same consent.

3. Summary of Expert P's views (researcher's record)

3.1 The narrow rehabilitation window. Elderly patients who enter rehabilitation after surgical intervention or recovery side-effects generally follow the prescribed guidelines at first — but the window is very narrow. If the rehabilitation does not deliver immediate or short-term improvement they normally give up: typically within a couple of weeks and not more than two months; sometimes within two weeks or after only a few attendances. The researcher noted this timing converges with the programme's own first observations of early disengagement.

3.2 Why they give up. Expert P identified the drivers in his caseload: (1) loss of faith in further improvement, connected to their sense of aging and approaching the end of life; (2) abandonment is mostly NOT caused by the difficulty of the health technology — lack of rewards, slow improvement and long time-to-results are the common challenges, particularly for the elderly group.

3.3 Necessary complexity. Simple systems can work, but some clinical assessment and rehabilitation cannot be made simple: balance abilities, fall-risk assessment, and mental and cognitive evaluation require complicated systems. He therefore accepted that a gamified system may help engagement and retention in using such health technology — gamification as the means of keeping necessary complexity bearable.

3.4 The infantilisation risk. Some games are far too simple: elderly users may not find them interesting enough and feel the game is childish. Challenge and presentation must be adult-appropriate.

3.5 Demographic differences and the ego effect. Different elderly demographic groups perform quite differently. The higher-educated group with better financial conditions are the key regular users of advanced health technology. But they behave similarly to others when the game's rewards become difficult to obtain — and their egos are typically higher than those of elderly people from general working-class backgrounds. In particular, if they are OBSERVED failing while playing, they will probably stop immediately. Failure witnessed by others is a stronger stopping trigger than failure alone.

3.6 The reward contradiction. Game rewards are critical particularly for general working-class elderly, whose capacity to accept health technology is far lower than that of the educated group. Expert P acknowledged this as a contradiction — the group that most needs rewards is hardest to reach with the technology, while the group the technology reaches is most sensitive to visible failure. He suggested an AI-designed game may help resolve it, while stating plainly that he did not know whether it would work.

3.7 Willingness to trial. He would like to try such a system in practice — a forward-validation prospect, not a finding.

3A. Follow-up interview (early August 2026)

3A.1 Reaction to the actual game. Expert P was asked to play the study's skiing coordination game. His judgement: the game is suitable for FRA; and if it were installed in the (FRA) system he would come back to play it regularly. This is the panel physiotherapist's direct assessment of the artifact itself — and his stated regular-return intention mirrors the study's D2 intention construct.

3A.2 Fall-risk assessment in Hong Kong. FRA is critical for elderly people, yet Hong Kong does not conduct this assessment regularly, because the community does not see its importance. Advertising and promotion may play a part, but lack of reference and difficulty of use could be a key reason. He also restated a point from the original interview that the researcher had not

previously recorded: health technology must be easy to use, otherwise it is very difficult to promote — alongside economic factors such as pricing and professional fees.

3A.3 Restatement on failure and complexity. Elderly people are not used to failing when practising with health technology — balance checking, for example, is a complicated system, so very few elderly people are willing to join the assessment at all. Among those who do try, finding the technology difficult to use, or slow to show improvement, leads to lost interest and engagement — and they never come back. This is very common where the procedure starts complicated.

Reconciliation note (researcher's analysis, kept distinct from Expert P's words): 3A.2-3A.3 nuance the original statement 3.2 that abandonment is "mostly not the difficulty of the health tech". Read together, his account is two-staged: difficulty and complexity act at ENTRY (they deter joining and can end engagement in the first encounters), while reward scarcity and slow visible progress drive abandonment among those who have engaged. Both statements are preserved verbatim above; the two-stage reading is the researcher's.

4. Relation to the thesis

(a) FIDS timing (§5.5): the two-week-to-two-month abandonment window from an independent clinical caseload converges with the programme's observed early-disengagement timing. (b) Attribution (§5.6, §8.5): abandonment is not driven by technology difficulty — reward scarcity, slow visible progress and hopelessness about improvement dominate; the aging-and-mortality component enriches the FIAS self-concept mechanism. (c) The avoided middle (§8.6B, Figure 8.1): balance, fall-risk and cognitive assessment cannot be simplified away — supporting the thesis argument that the measuring middle ground must be made bearable rather than avoided. (d) Design implications (§7.5): the infantilisation warning and the observed-failure/ego effect support adult-calibrated challenge and private, unsupervised play — the Stage-3 deployment choice — and add a social-exposure moderator to the failure mechanism. (e) Demographic convergence: his educated/financially-comfortable adopter profile matches the fourth consultation's independent account, and his reward-contradiction sharpens the adaptive-learning framework's calibration problem. As with the whole panel, these views are interpretation and practice validation; they are not additional empirical evidence and do not substitute for sample size. (f) From the follow-up: the panel physiotherapist judged the actual game suitable for FRA and stated a regular-return intention — the strongest artifact-level validation in the panel record, though a single expert judgement, not user evidence. (g) His account of Hong Kong's irregular fall-risk assessment (importance not perceived; ease-of-use and promotion barriers) strengthens the thesis's problem statement from local practice. (h) The entry-versus-retention two-stage reading (3A note) maps onto the programme's FIDS/FIAS distinction: ease-of-use governs whether engagement begins; reward and visible progress govern whether it survives.

5. Records checklist

Archived: signed + countersigned consent form (4 Aug 2026); WhatsApp transmittal screenshot (including the participant's first incomplete return, corrected the same evening). This summary is

drafted from the author's texted account of 4 Aug 2026; original written notes may be added to the confidential file as optional strengthening.